and to initiate appropriate preventive measures. The diagnostic cycle begins when the clinician takes a microbiological sample and ends when a clinician receives the laboratory report and uses the information to manage the condition (Figure 12.3).

The steps in diagnostic cycle are

1. Clinical request and provision of clinical information.
2. Collection and transport of appropriate specimens.
3. Laboratory analysis.
4. Interpretation of microbiology report and use of the information.

Specimen Collection and transport: It is important to collect the specimen appropriately and protect it from contamination. Transport media are used that are compatible with the organism believed to be present in the clinical sample. Quality of patient specimens and their transport to the laboratory is important.

Infections and samples used

Respiratory tract infections: Nasal and bronchial washings, throat and nasal swabs, sputum.

Eye infections: Conjunctival swab or scraping.

Wound infections: Pus, skin scraping, wound swap.

Gastrointestinal infections: Stool, rectal swabs.

Genital infections: Vesicle fluid or swab.

Urinary tract infections: Urine.

Blood borne infections: Blood.

Nervous system infections: Cerebrospinalfluid (CSF).

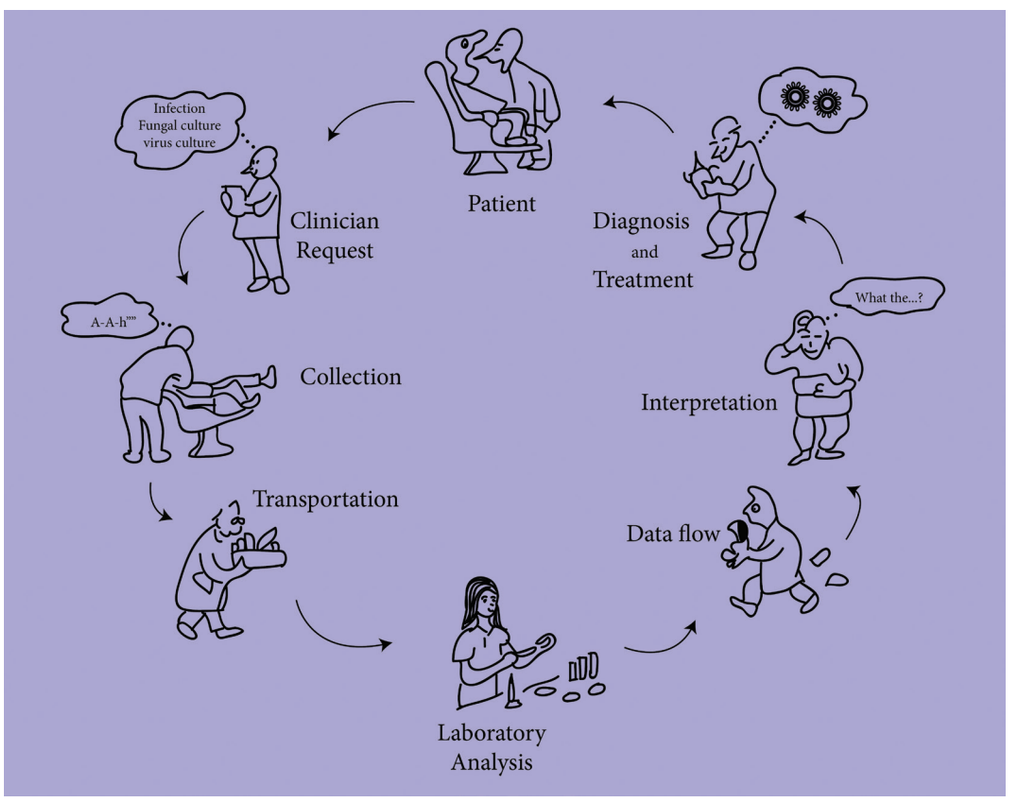


Figure 12.3: The steps in diagnostic cycle

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